

1. Administrative & Study Identification

**Official Study Title:** A Phase 2 Clinical Trial of Nivolumab, or Nivolumab Combinations in Recurrent and Metastatic Microsatellite Instability High (MSI-H) and Non-MSI-H Colon Cancer **Plain Language Study Title:** A Study of Nivolumab Alone or Nivolumab Combination Therapy in Colon Cancer That Has Come Back or Has Spread **Short Title/Acronym:** CheckMate 142 **Protocol Number:** CA209-142 **NCT Number:** NCT02060188 **Trial Status:** COMPLETED (Study has been completed) **Sponsor/Company Name:** Bristol-Myers Squibb **Investigational Product:** Nivolumab (Opdivo), Ipilimumab (Yervoy), Relatlimab, Cobimetinib, Daratumumab **Phase of Development:** Phase II **Medical Monitor:** Bristol-Myers Squibb Medical Operations Lead

---

2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To evaluate the efficacy of Nivolumab as a monotherapy or in combination with other anti-cancer drugs (e.g., Ipilimumab, Relatlimab) by determining the Objective Response Rate (ORR) per RECIST v1.1 per Investigator Assessment in patients with recurrent or metastatic colon cancer. **Secondary Objectives:** To assess Objective Response Rate (ORR) by RECIST v1.1 per Independent Review Committee (IRC), Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DOR), Disease Control Rate (DCR), and the incidence and severity of treatment-related adverse events.

**2.2 Background & Rationale** To address the critical unmet medical need for durable and effective therapies in patients with recurrent or metastatic colorectal cancer (mCRC). Specifically, tumors with deficient mismatch repair (dMMR) or high microsatellite instability (MSI-H) possess high mutational burdens and significant immune cell infiltration, making them prime targets for immune checkpoint inhibitors targeting PD-1 (nivolumab), CTLA-4 (ipilimumab), and LAG-3 (relatlimab) pathways.

---

3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Multiple parallel-cohort (Non-randomized) **Blinding:** Open-label **Randomization:** N/A (Parallel Assignment)

**3.2 Planned Enrollment & Duration Targeted Enrollment:** 385 participants across multiple sequential and parallel cohorts

4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 1. 2. Histologically confirmed recurrent or metastatic colorectal cancer. 3. Measurable disease per RECIST v1.1. 4. Microsatellite instability expression (MSI-H) or mismatch repair deficiency (dMMR) detected by an accredited laboratory. 5. Participants enrolled into the C3 Cohort must have not had treatment for their metastatic disease.

**4.2 Exclusion Criteria** 1. Prior treatment with an anti-PD-1, anti-PD-L1, anti-PD-L2, anti-CTLA-4 antibody, or any other antibody or drug specifically targeting T-cell co-stimulation or immune checkpoint pathways. 2. Participants with an active, known, or suspected autoimmune disease. 3. A condition requiring systemic treatment with either corticosteroids or other immunosuppressive medications within 14 days of study drug administration. 4. Active brain metastases or leptomeningeal metastases are not allowed. 5. Prior malignancy active within the previous 3 years except for locally curable cancers.

---

5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Multiple dosing arms, notably including Nivolumab 3 mg/kg IV every 2 weeks (monotherapy) OR Nivolumab 3 mg/kg combined with Ipilimumab (ATC Code: L01FX04) 1 mg/kg IV every 3 weeks for 4 doses, followed by Nivolumab maintenance. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until unacceptable toxicity, radiographic progression, or patient withdrawal.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard palliative care and symptom management therapies. **Prohibited:** Concurrent anti-cancer therapy, investigational agents, or systemic immunosuppressive corticosteroids (>10 mg prednisone equivalent).

---

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---  
| :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor biopsy/Biomarker confirmation (MSI-H/dMMR), Baseline Scans | | **Baseline** | Day 0 | Cohort Assignment, First Dose, Safety Labs | | **Interim** | Every 2 to 3 Weeks | IV Infusions, Toxicity Assessments, Efficacy Scans (every 6-12 weeks) | | **End of Study** | Variable | End of Treatment Visit, Survival Follow-Up, Safety Tracking |

---

## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Objective Response Rate (ORR) by Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 Per Investigator Assessment. ORR is defined as the percentage of randomized participants who achieve a best overall response (BOR) of confirmed complete response (CR) or partial response (PR).  
**Measurement Method:** Investigator Assessment per RECIST v1.1.

**7.2 Secondary & Exploratory Endpoints** 1. Objective Response Rate (ORR) by RECIST v1.1 Per Independent Review Committee (IRC) 2. Progression-Free Survival (PFS) and Overall Survival (OS) 3. Duration of Response (DOR) and Disease Control Rate (DCR) 4. Incidence of adverse events (AEs) and immune-related adverse events (irAEs)

---

## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Routine collection via clinical evaluation, laboratory tests, and patient reporting at each visit, categorized by CTCAE criteria. **Serious Adverse Events (SAEs):** Accelerated reporting timeline (within 24 hours of investigator awareness) for severe or life-threatening immune-related adverse events. **Data Monitoring Committee (DMC):** Independent safety review and data monitoring committees evaluate ongoing toxicity and benefit-risk profiles across all cohorts.

---

## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** All treated patients (Safety/Intent-to-Treat Set) who received at least one dose of the study drug.  
**Sample Size Justification:** Cohorts powered to detect a meaningful ORR margin over historical control rates for chemo-refractory or untreated mCRC. Target enrollment is 385 patients. **Handling of Missing Data:** Participants with missing post-baseline tumor assessments are generally treated as non-responders in the ORR analysis; survival endpoints evaluated via Kaplan-Meier methodology.

---

## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.  
**Monitoring Plan:** Standard clinical oncology monitoring, incorporating independent radiological blinding reviews for primary endpoint integrity. **Audit Trail:** Secure eCRF systems, protocol amendment tracking, and centralized pathology/biomarker testing validation.

---

## 11. Study Identifiers & Governance

**Primary ID:** CA209-142 **Registry Number:** NCT02060188 **Posting ID:** 122988495005327265 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** CheckMate 142 **Official Regulatory Title:** A Phase 2 Clinical Trial of Nivolumab, or Nivolumab Combinations in Recurrent and Metastatic Microsatellite Instability High (MSI-H) and Non-MSI-H Colon Cancer

---

## 12. Clinical Framework (Colorectal Cancer Specific)

**Primary Condition:** Metastatic Colorectal Cancer (mCRC) **Specific Conditions/Names:** Rectal Cancer (Preferred Name: Rectal Carcinoma) **Preferred UMLS Name:** Advanced colorectal cancer **Disease Areas:** Microsatellite Unstable Colorectal Cancer, Microsatellite Stable Colorectal Cancer, Mismatch Repair Proficient Colorectal Cancer, Mismatch Repair Deficient Colorectal Cancer **Diagnosis Threshold:** Microsatellite Instability-High (MSI-H) or Mismatch Repair Deficient (dMMR) confirmed by accredited lab **Medical Methodology:** Immune Checkpoint Inhibition **Optimization Target:** Objective Response Rate (ORR) / Tumor Regression

---

## 13. Study Procedures & Methodology

**Management Pathway:** RECIST v1.1 standardized oncology tumor assessment **Education Protocol (HCP):** Clinical investigator training on immune-related adverse event (irAE) recognition and algorithm-based management **Education Protocol (Patient):** Immunotherapy toxicity reporting and early intervention protocols **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) / Independent Radiographic Review Committee (IRC)

---

## 14. Observation & Follow-Up Schedule

**Total Duration:** Minimum 2 years of active treatment or until disease progression, plus long-term survival follow-up **Onsite Visit Cadence:** Every 2 to 3 weeks for dosing/infusion (depending on exact combination cohort) **Remote Monitoring Frequency:** Safety follow-up extending for 100 days post-last dose **Checklist Review Interval:** Radiographic imaging every 6 weeks for the first 24 weeks, and then every 12 weeks thereafter

---

## 15. Sign-off & Approval

|                        |        |           |               |     |     |     |     |
|------------------------|--------|-----------|---------------|-----|-----|-----|-----|
| Role                   | Name   | Signature | Date          | :-- | :-- | :-- | :-- |
| Principal Investigator | [Name] | ____      | [DD-MMM-YYYY] |     |     |     |     |

Clinical Operations Lead | [Name] | \_\_\_\_ | [DD-MMM-YYYY] | | Lead  
Statistician | [Name] | \_\_\_\_\_ | [DD-MMM-YYYY] |